

Retinal Review Workbench

Clinician User Manual

Retinal Review Workbench

2026-09-23

Table of contents

1	Retinal Review Workbench	3
1.1	Document control	3
1.1.1	Revision history	3
1.2	Two-minute workflow	4
2	Purpose and scope	5
2.0.1	Intended users	5
2.0.2	Supported input intent	5
2.0.3	System overview	5
2.0.4	Safety and interpretation	5
2.0.5	Navigation map	6
3	Worklist and case setup	7
3.0.1	Start the workstation	7
3.0.2	Confirm Image	7
3.0.3	Worklist exceptions	9
4	Review workspace	11
4.0.1	Review the retinal image	11
4.0.2	RETFound suggestion	11
4.0.3	PRISM overlays	13
4.0.4	Models & Audit	13
5	Clinician review and annotations	14
5.0.1	Confirm DR Grade	14
5.0.2	Edit Annotations	14
5.0.3	Correct an AI ROI class	17
5.0.4	Remove an AI ROI	18
5.0.5	Reviewer preference and draft state	18
5.0.6	Previous, Next, and hints	18
6	Dataset and audit surfaces	19
6.0.1	Dataset readiness	19
6.0.2	Export the manifest	19
6.0.3	Models & Audit	21
6.0.4	Optional CVAT	21
7	Troubleshooting and quick reference	23
7.0.1	Privacy and safety limitations	23
7.0.2	Quick reference	23

1 Retinal Review Workbench

Clinician User Manual



This manual describes the final clinician workflow for **Retinal Review Workbench**. It is intended for clinician onboarding, hospital/internal demonstration, clinical research workflow documentation, and software audit.

WARNING — clinical scope

This software provides model-assisted evidence for clinician review. It does not autonomously establish diagnosis, disease truth, referral, or treatment decisions. Human review remains authoritative.

1.1 Document control

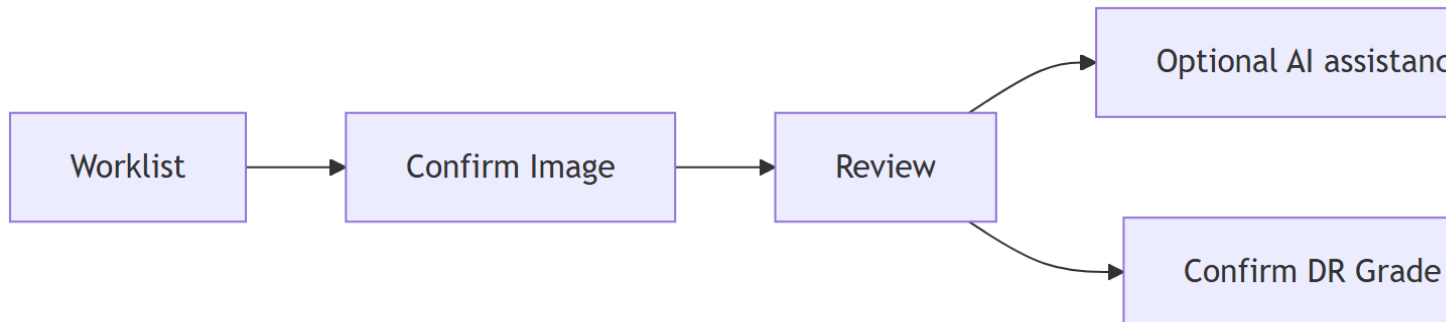
Field	Value
Document title	Retinal Review Workbench — Clinician User Manual
Document ID	RRW-CLIN-UM-001
Software version	0.6.0
Manual version	1.0
Status	Controlled release documentation
Date	2026-09-23
Owner	Retinal Review Workbench maintainers

1.1.1 Revision history

Version	Date	Change
1.0	2026-09-23	Rebuilt as a Quarto Book for the simplified clinician workflow and v2 dataset manifest.

1.2 Two-minute workflow

1. In **Worklist**, select a case and complete **Confirm Image**.
2. In **Review**, inspect the retinal image and run optional analysis if the Model API is available.
3. Toggle or filter PRISM overlays as needed. No per-detection confirmation is required.
4. In **Clinician Review**, choose the final grade and select **Confirm DR Grade**.
5. In **Edit Annotations**, make only the corrections or additions needed, then select **Confirm Annotation** when the active annotation set is reviewed.
6. In **Datasets**, check **DR-ready** and **Lesion-ready** separately before export.



! IMPORTANT — three milestones

The three confirmation actions are case-level workflow milestones. ROI correction and removal are optional edits; they are not a queue of detections that must all be resolved.

2 Purpose and scope

2.0.1 Intended users

This manual is for clinicians and reviewers operating the local review workstation, dataset operators preparing research exports, and hospital operators supporting the workstation and Model API.

2.0.2 Supported input intent

The workstation supports JPEG, PNG, TIFF, and supported single-frame ophthalmic fundus DICOM. Ophthalmic DICOM uses the existing display and provenance path. MRI, OCT, PACS objects, non-fundus DICOM, unsupported SOP classes, multi-frame objects, and untested transfer syntaxes are outside model support.

When a recognized DICOM object is not a supported fundus modality, Worklist reports **Unsupported modality**. This is distinct from a generic decoder or image-read failure.

2.0.3 System overview

The review workstation uses local Workspace state and calls a provider-neutral Model API when analysis is requested. The workstation does not load model weights in the **review** profile.

Component	Operating role
Review workstation	Worklist, image display, clinician decisions, annotations, audit display, and export.
RETFound	Five-class DR grade suggestion through retfound-aptos5 .
PRISM-DR	Optional lesion ROI suggestions through prism-dr-5fold .
Model API	Remote hospital service for the existing inference contracts.
CVAT Online	Optional dense annotation and geometry workflow.

2.0.4 Safety and interpretation

WARNING — model interpretation

RETFound and PRISM-DR output is model-assisted evidence. A displayed confidence or score is not a calibrated clinical probability. An empty PRISM result does not prove that lesions are absent.

The final clinician grade, annotation state, and exception decision are human-authored. Original AI evidence remains available in **Models & Audit** and the dataset provenance fields.

2.0.5 Navigation map

- **Worklist:** admission state, context resolution, exclusions, and case entry.
- **Review:** image-first inspection, optional analysis, overlays, and compact model summaries.
- **Clinician Review:** final DR grade and escalation.
- **Edit Annotations:** human annotation work and optional AI ROI correction/removal.
- **Datasets:** task-specific readiness and export.
- **Models & Audit:** read-only technical provenance.
- **Settings:** Workspace and workstation preferences.

3 Worklist and case setup

3.0.1 Start the workstation

3.0.1.1 Goal

Open the clinician UI and establish the local Workspace used for the review session.

3.0.1.2 Before you start

The operator must have completed the workstation installation and built the frontend. If model assistance is required, the operator must configure the hospital Model API connection privately.

3.0.1.3 Steps

1. Start `START.cmd`, or start the documented review profile manually.
2. Open `http://127.0.0.1:8000/app/`.
3. Open **Settings** and create or select a Workspace.
4. Confirm the input folder, output folder, and SQLite review database shown for the Workspace.
5. Return to **Worklist** and select **Scan input folder**.

3.0.1.4 Expected result

Admitted files appear with plain-language status. Original source files remain immutable.

3.0.1.5 What the system records

The Workspace catalog stores profile paths and case state. Admission stores source identity, dimensions, modality decision, quality state, and source hash.

3.0.2 Confirm Image

3.0.2.1 Goal

Confirm the image context needed before downstream analysis and review.

3.0.2.2 Before you start

Open a case with **Confirm Image** available. Treat any filename or resolver suggestion as evidence only.

Retinal Review

Workbench

WORKSPACE

Worklist

Datasets

Review

Models & Audit

ACTIVE WORKSPACE

Demo

Manage workspace

CONFIGURATION

Settings

Public & synthetic only

Research use v0.0.2

Worklist

Public & synthetic cases for clinician review

WORKLIST

Admitted retinal images for clinician review

Worklist

Review admitted images, resolve patient and eye identity, and keep AI assistance separate from the clinician decision.

Scan input folder

Refresh

Search filename or patient key

Filters

Sort: Filename

Cases

Patients

Search by filename or pseudonymous patient key

34 cases - 27 need attention

IMAGE	PATIENT / EYE	AI RESULT	REVIEW	ACTION
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	All unavailable	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	All unavailable	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	PAT001 - Eye side not specified Confirmed	Grade 2 Suggestion available	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Grade 2 Suggestion available	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	MG10 - Left Auto-linked	Not analyzed	Pending review	Review
	MG10 - Left Auto-linked	Not analyzed	Pending review	Review
	MG14 - Right Auto-linked	Not analyzed	Pending review	Review
	MG16 - Right Auto-linked	Not analyzed	Pending review	Review
	PAT0002 - Left Auto-linked	Not analyzed	Pending review	Review
	PAT0002 - Right Auto-linked	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Grade 2 Suggestion available	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review

AI suggestions are optional visual evidence; clinician review remains authoritative.

Suggestions support review. They do not establish a diagnosis or autonomous referral.

Figure 3.1: Worklist orientation

3.0.2.3 Steps

1. Select **Confirm Image** in the Worklist action cell.
2. Review the image name and image status.
3. Enter or clear the **Pseudonymous patient key**. Leave it empty when the case should remain unlinked.
4. Set **Eye** to **Left**, **Right**, or **Unknown**.
5. Enter **Reviewer name**, or use the workstation default if appropriate.
6. Optionally select **Use as default reviewer on this workstation**.
7. Select **Confirm Image**.

3.0.2.4 Expected result

The case context and image review timestamp are saved as one milestone. A supported fundus image can proceed to **Review**.

3.0.2.5 What the system records

The system records patient/eye resolution, image admission review, reviewer, timestamp, and the CONFIRM_IMAGE audit event. It does not infer authenticated identity.

! IMPORTANT — shared workstations

The default reviewer is a browser-local convenience. Change or clear it when the workstation user changes. Previously recorded reviewer names are not rewritten.

3.0.3 Worklist exceptions

Use the visible exception action when a case is not suitable for routine review. An MRI or other non-fundus DICOM must remain **Unsupported modality**. Do not send it to RETFound or PRISM-DR.

Retinal Review

Workbench

WORKSPACE

Worklist

Datasets

Review

Models & Audit

ACTIVE WORKSPACE

Demo

Manage workspace

CONFIGURATION

Settings

Public & synthetic only

Research use v0.0.0

Worklist

Public & synthetic cases for clinician review

WORKLIST

Admitted retinal images for clinician review

Review admitted images, resolve patient and eye identity, and keep AI assistance separate from the clinician decision.

Scan input folder

Refresh

Search filename or patient key

Filters

Sort: Filename

Cancel

Patients

Search by filename or pseudonymous patient key

34 cases - 27 need attention

IMAGE	PATIENT / EYE	AI RESULT	REVIEW	ACTION
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	All unavailable	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	All unavailable	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	PAT001 - Eye side not specified Confirmed	Grade 2	Suggestion available	Pending review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Grade 2	Suggestion available	Pending review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review
	MG10 - Left Auto-linked	Not analyzed	Pending review	Review
	MG10 - Left Auto-linked	Not analyzed	Pending review	Review
	MG14 - Right Auto-linked	Not analyzed	Pending review	Review
	MG16 - Right Auto-linked	Not analyzed	Pending review	Review
	PAT0002 - Left Auto-linked	Not analyzed	Pending review	Review
	PAT0002 - Right Auto-linked	Not analyzed	Pending review	Review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Grade 2	Suggestion available	Pending review
	Patient not linked Eye side needs confirmation Link patient and confirm eye	Not analyzed	Pending review	Review

AI suggestions are optional visual evidence; clinician review remains authoritative.

Suggestions support review. They do not establish a diagnosis or autonomous referral.

Figure 3.2: Confirm Image and reviewer

4 Review workspace

4.0.1 Review the retinal image

4.0.1.1 Goal

Inspect the original retinal image and decide whether optional model evidence is useful.

4.0.1.2 Before you start

The case must be admitted as a supported retinal input. Resolve or confirm the context as required by the Worklist state.

4.0.1.3 Steps

1. Open the case and select **Review**.
2. Use the image viewer controls for fit, zoom, pan, full-screen inspection, or coordinate inspection as needed.
3. Read the patient/eye context shown above the image.
4. If model assistance is needed and available, select **Analyze**.
5. Use **Show AI suggestions** to show or hide PRISM overlays.
6. Use the lesion filter to show **All lesion classes**, **MA** · **Microaneurysm**, **HE** · **Hemorrhage**, **EX** · **Hard exudate**, or **SE** · **Soft exudate**.

4.0.1.4 Expected result

The image remains the largest and primary review surface. The right side shows a compact DR assessment, lesion suggestion counts, and model/runtime status. An untouched ROI may display a class and score such as HE · 0.90.

4.0.1.5 What the system records

Analysis records the existing model outputs, model identifiers, inference metadata, source hash, and analysis lineage. Toggling or filtering overlays does not alter the model result.

i NOTE — no evidence queue

PRISM detections are optional visual assistance. Review completion does not depend on confirming, rejecting, or reducing an unresolved detection count to zero.

4.0.2 RETFound suggestion

RETFound provides a five-class DR grade suggestion and model score. It is not automatically confirmed. Use **Clinician Review** to select the final grade.

Retinal Review
Workbench

WORKSPACE

Worklist

Datasets

Review

Models & Audit

ACTIVE WORKSPACE

Demo

Manage workspace

CONFIGURATION

Settings

Public & synthetic only
Research use - v0.6.0

Review

Inspect optional model evidence before clinician sign-off

WARNINGS

Review

SYNTH_001 - optional model evidence for clinician inspection

← Previous

33 of 34

Next →

Next action: Confirm image

Dismiss

Retinal preview

640 × 480px - CFP

Scroll to zoom - drag to pan - double-click to fit

100%

Fit

Viewer 110%

All overlay geometry remains in original image pixels.

AI suggestion

Human annotation

Show AI suggestions

All lesion classes

Active overlays 1 of 1 raw AI suggestions

Microaneurysm

Hemorrhage

Hard exudate

Soft exudate

Image ID

Dimensions

Source

SYNTH_001

640 × 480px

Generated workflow fixture

Model / runtime status

Status reported by the existing model metadata contract.

retfound-aptos5

runtime not reported

CC-BY-NC-4.0; research/non-commercial only

Model setup required

prism-dr-5fold

runtime not reported

Research model; no scientific performance claim

Model setup required

Model notes available

Technical model notes are available in Models & Audit. Review remains available; confirm the image and human decision.

Analysis

Run both existing remote inference contracts on this admitted image.

Actual returned results are shown below.

Analyze again

DR assessment

System-predicted DR grade

Grade 2

Model confidence

70.0%

Model: mock-global - synthetic-v1

Lesion suggestions

AI lesion suggestions

1

Suggested classes

Microaneurysm

1

Raw AI suggestions by class. Optional visual assistance; no per-detection action is required.

Model: mock-lesion - synthetic-v1

Edit annotations

Clinician review

Suggestions support review. They do not establish a diagnosis or autonomous referral.

Figure 4.1: Review workspace

4.0.3 PRISM overlays

PRISM-DR provides bounded visual lesion suggestions. The compact summary describes raw AI suggestions by lesion class; it is not a count of clinician-reviewed annotations. A human correction is shown without reassigning the original AI score to the new class.

4.0.4 Models & Audit

Open **Models & Audit** for read-only model IDs, revisions, runtime status, inference events, detection counts, source/analysis SHA-256 values, and annotation provenance. This page is for inspection and audit, not another clinical task list.

5 Clinician review and annotations

5.0.1 Confirm DR Grade

5.0.1.1 Goal

Record the clinician's final DR grade without requiring a separate "accept" or "mark incorrect" workflow.

5.0.1.2 Before you start

Review the image and the optional RETFound suggestion. The final grade must be selected by the clinician.

5.0.1.3 Steps

1. Open **Clinician Review**.
2. Review **AI suggestion** when available.
3. Select a value from **Final DR grade**.
4. Check or edit **Reviewer name**.
5. Add an optional **Remark**.
6. Select **Confirm DR Grade**. Select **Escalate** for an exception path instead.

5.0.1.4 Expected result

The final grade is saved as **AI_ACCEPTED** when it matches the AI suggestion, **AI_CORRECTED** when it differs, or **MANUAL** when no AI grade was available. The human grade is authoritative.

5.0.1.5 What the system records

The system records grade, review source, reviewer, timestamp, revision, and review history. It does not silently accept a grade merely because the page was opened.

5.0.2 Edit Annotations

5.0.2.1 Goal

Add or correct the active annotation set when the clinician notices an annotation that needs a human-authored change.

5.0.2.2 Before you start

Open **Edit annotations** from Review. Geometry editing is available for human annotations in original image pixel coordinates. Advanced dense correction may use the optional CVAT workflow.

Retinal Review
Workbench

WORKSPACE

Worklist

Datasets

Review

Models & Audit

ACTIVE WORKSPACE

Demo

Manage workspace

CONFIGURATION

Settings

Public & synthetic only
Research use - v0.6.0

Clinician Review

Human sign-off and review record

WARNINGS

SYNTH_001 - human sign-off and review record

← Back to Review

Back to Worklist

← Previous

33 of 34

Confirm DR Grade & Next

Next action: Confirm image

Dismiss

Review summary

AI output is a suggestion; this page records the clinician decision.

Scroll to zoom - drag to pan - double-click to fit

Q

100%

Q

Fit

⌕

⌕

Viewer 102% All overlay geometry remains in original image pixels. ☒ AI suggestion ☐ Human annotation

AI-predicted grade	Human annotations	AI lesion suggestions
Grade 2	0	1

Current review state

Pending review

Clinician sign-off

Save a human review action with an optional remark.

Reviewer name *

☐ Use as default reviewer on this workstation

AI suggestion: Grade 2

Final DR grade

Grade 2

▼

Remark

Confirm DR Grade

Escalate

Review record

Persisted in the existing SQLite case store.

No clinician review saved yet.

Suggestions support review. They do not establish a diagnosis or autonomous referral.

Figure 5.1: Clinician grade confirmation

15

5.0.2.3 Steps

1. Select **Edit annotations**.
2. Keep **AI suggestions** shown when comparing the model evidence, or hide them to reduce visual density.
3. Select an annotation tool and choose the canonical lesion class.
4. Draw, move, resize, lock, or delete a human annotation as needed.
5. Select **Save draft** to persist a non-final draft, or **Save & Next** to save the same draft and open the next case.
6. Select **Confirm Annotation** only after reviewing the current active set.

5.0.2.4 Expected result

Human annotations remain separate from original AI evidence. **Confirm Annotation** records the current active set hash, reviewer, and timestamp. Later edits invalidate that confirmation until the set is confirmed again.

5.0.2.5 What the system records

Human shape geometry, canonical class, reviewer, timestamp, revision, active annotation-set hash, and case-level finality are retained. Empty annotations may be confirmed as reviewed; they are not evidence that lesions are absent.

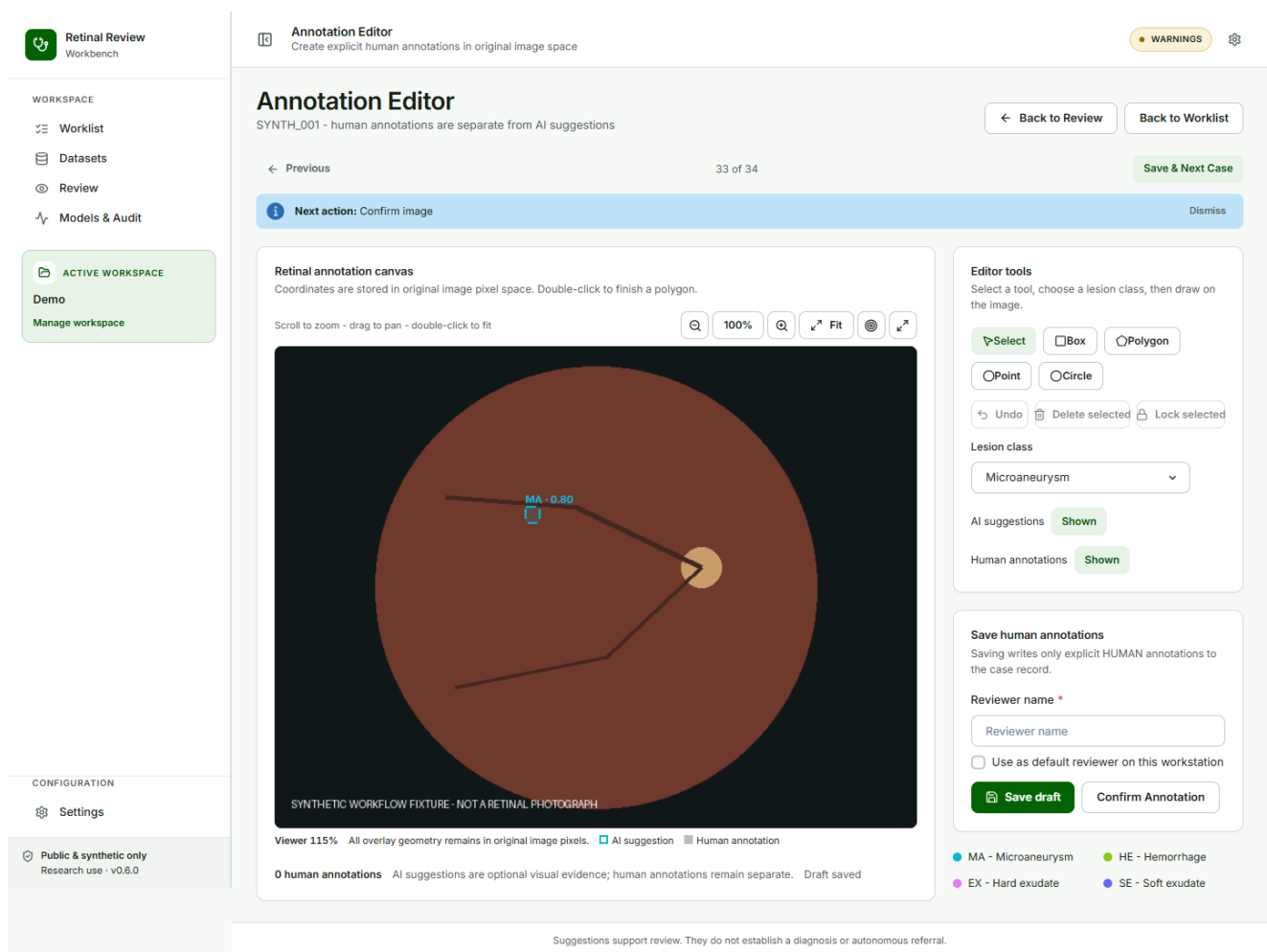


Figure 5.2: Annotation editor

5.0.3 Correct an AI ROI class

5.0.3.1 Goal

Correct an obvious model class without changing the original AI evidence.

5.0.3.2 Steps

1. In **Edit Annotations**, ensure **AI suggestions** are shown.
2. Select the AI ROI on the image.
3. Review **AI suggestion**, including its original class and score.
4. Choose a different value under **Change lesion class**.
5. Enter or confirm **Reviewer name** and optionally select **Use as default reviewer on this workstation**.
6. Select **Save class correction**.

5.0.3.3 Expected result

The overlay uses the reviewed class and indicates that it was clinician corrected. It does not display the original score as though it belonged to the corrected class. The detail remains available as AI suggestion: Hemorrhage · 0.90.

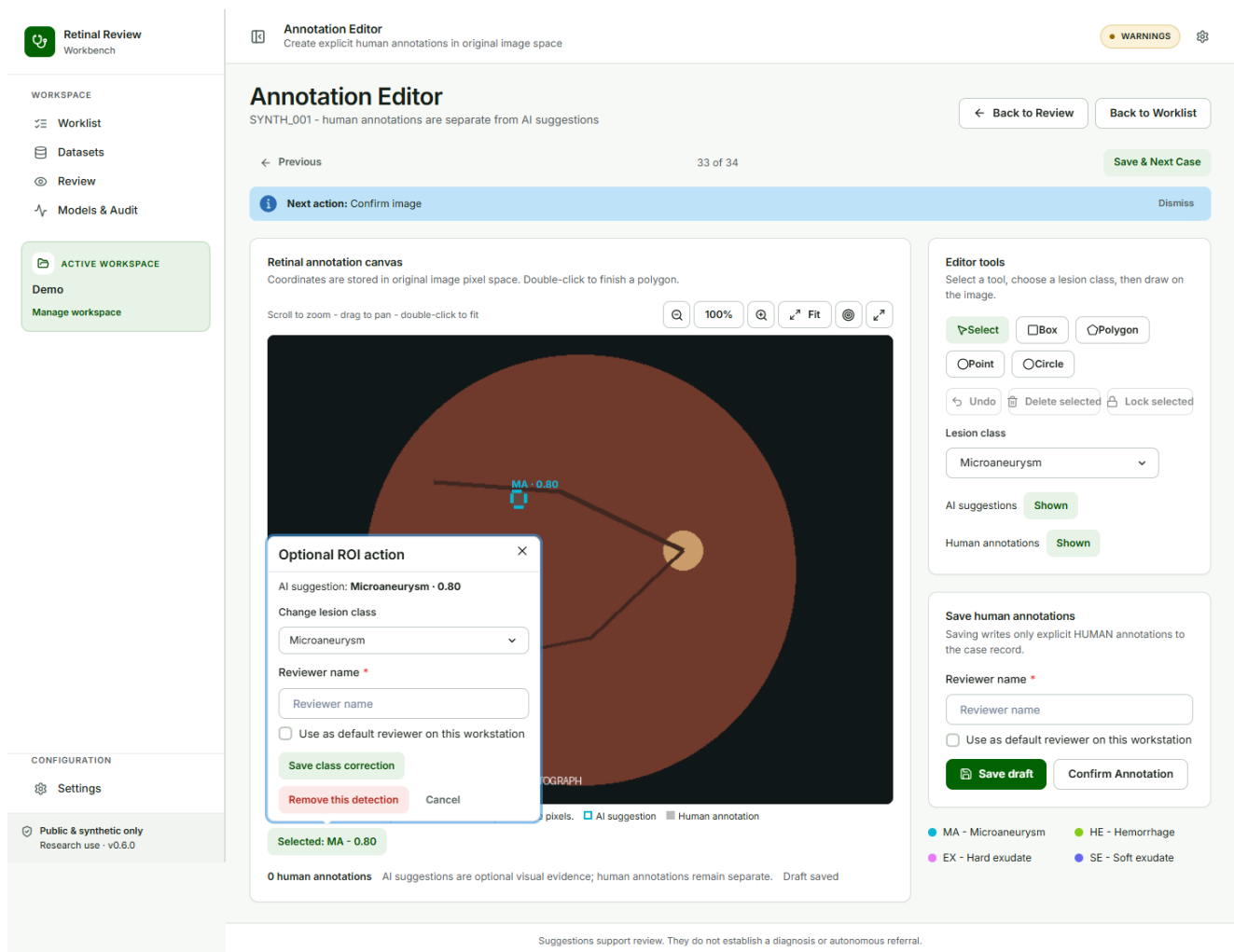


Figure 5.3: AI ROI correction popover

5.0.4 Remove an AI ROI

Select the ROI, review its original evidence, and choose **Remove this detection**. The detection is excluded from the active reviewed overlay and training annotation set, but its original detection identity, class, score, geometry, model, and provenance remain auditable.

WARNING — score meaning

If AI predicted Hemorrhage · 0.90 and the reviewer changes the class to Microaneurysm, do not describe the result as Microaneurysm · 0.90. The score belongs to the original AI suggestion only.

5.0.5 Reviewer preference and draft state

Use **Default reviewer** in **Settings** to set or clear the workstation value. New or unattributed forms prefill it; an existing recorded reviewer remains unchanged.

In Edit Annotations, the quiet status line reports **Saving draft**, **Draft saved**, **Unsaved changes**, or **Save failed — retry**. Autosave uses the existing annotation persistence path and never confirms the set. Leaving while changes are unsaved, in flight, or failed opens a guard.

5.0.6 Previous, Next, and hints

Use **Previous** and **Next** on Review, Clinician Review, and Edit Annotations. Navigation does not save or confirm. **Next action** is guidance only and can be disabled from the hint close control or re-enabled in **Settings**.

6 Dataset and audit surfaces

6.0.1 Dataset readiness

6.0.1.1 Goal

Determine whether a case is ready for DR-grade work, lesion work, both, neither, or exclusion.

6.0.1.2 Steps

1. Open **Datasets**.
2. Select **DR-ready**, **Lesion-ready**, **Needs review**, or **Excluded**.
3. Read the task-specific status in the case row.
4. Open **Models & Audit** when technical lineage or model provenance needs inspection.

6.0.1.3 Expected result

Classification and lesion readiness are shown separately. A final grade can make a case DR-ready without annotation confirmation. A confirmed annotation set can make a case Lesion-ready without a final DR grade.

6.0.2 Export the manifest

6.0.2.1 Before you start

Open an active Workspace and verify that the current case state and readiness are correct.

6.0.2.2 Steps

1. Select **Export manifest**.
2. Confirm the success message and output directory shown by the application.
3. Give the package to the approved dataset operator for downstream handling.

6.0.2.3 Expected result

The output contains `manifest.json`, `images.csv`, and `annotations.csv`. The manifest schema is `s4.dataset-manifest.v2` and records file hashes, source hashes, coordinate system, canonical taxonomy, confirmation states, task readiness, and provenance.

6.0.2.4 What the system records

Untouched AI, corrected AI, removed AI, human, and CVAT-imported annotations remain distinguishable. The export does not silently promote AI pseudo-labels to ground truth.

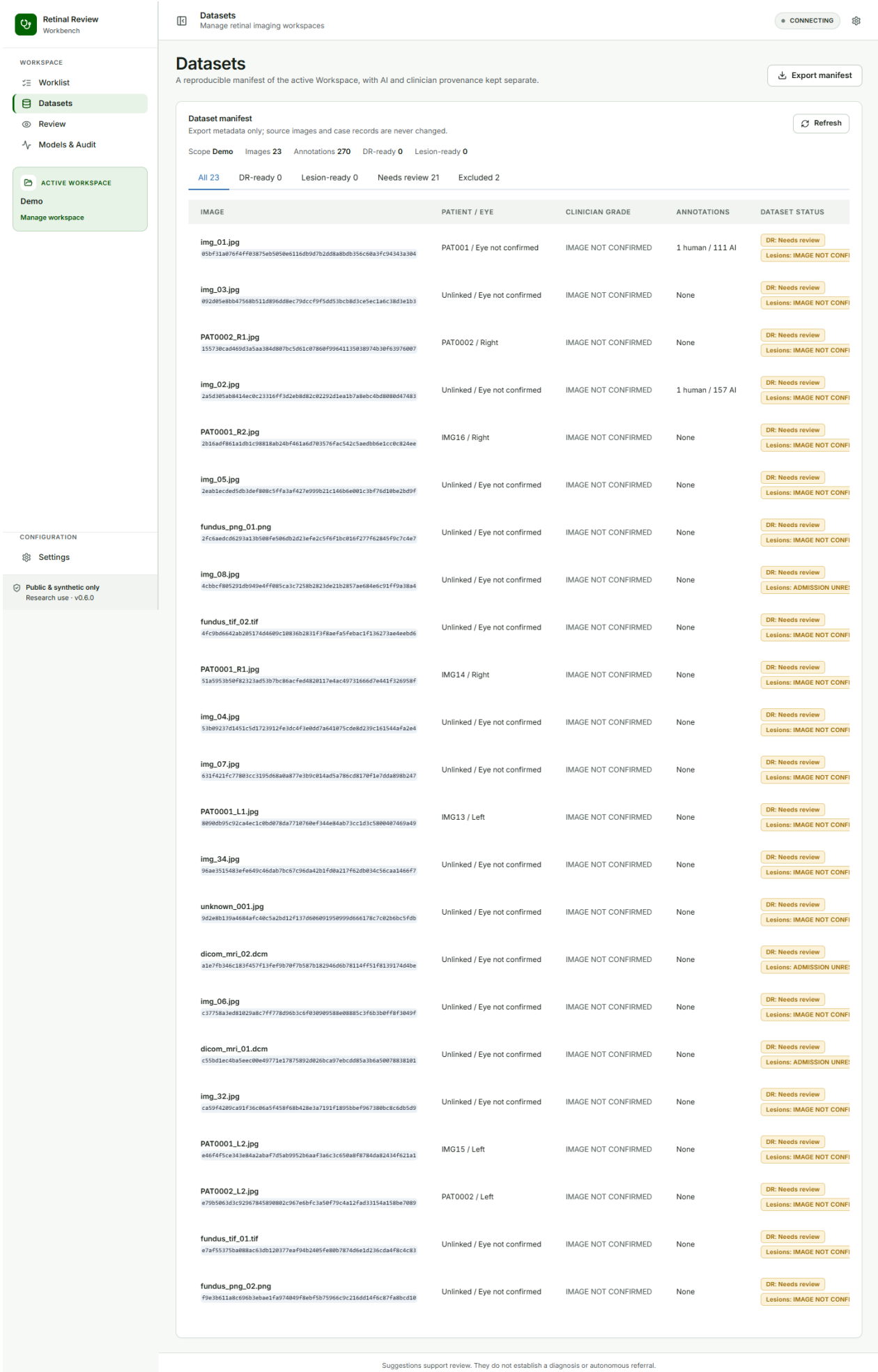


Figure 6.1: Dataset readiness

6.0.3 Models & Audit

Use this read-only surface to inspect:

- RETFound and PRISM-DR model identifiers, revisions, runtime status, and warnings;
- grade and lesion inference metadata and counts by class;
- source SHA-256 and analysis/derivative SHA-256 values;
- annotation source, correction/removal provenance, and finality state.

6.0.4 Optional CVAT

Use the configured CVAT Online integration only for dense annotation or geometry correction. The round trip preserves source and prediction hashes, validates the configured project and labels, and is idempotent for a known task. CVAT is optional and excluded from offline acceptance.

Retinal Review
Workbench

WORKSPACE

Worklist

Datasets

Review

Models & Audit

ACTIVE WORKSPACE

Demo

Manage workspace

CONFIGURATION

Settings

Public & synthetic only
Research use - v0.6.0

Models & Audit

Provider readiness, provenance, limitations

CONNECTING

Models & Audit

Read-only model metadata, case results, and bounded visual evidence

Model readiness

Metadata is read from the existing /v1/models contract; unavailable remote state is shown honestly.

retfound-aptos5

global - runtime not reported

CC-BY-NC-4.0; research/non-commercial only

ASSET_REQUIRED

prism-dr-5fold

lesion-roi - runtime not reported

Research model; no scientific performance claim

ASSET_REQUIRED

Case context

Choose a returned case to inspect its actual inference metadata and results.

SYNTH_001 - SYNTH_001

RETFound - Score-level decision context

No saliency explanation is produced by the current RETFound bridge.

Model id

mock-global

Revision

synthetic-v1

Task

global

Runtime / status

remote - ASSET_REQUIRED

Preprocessing

synthetic-fixture-v1

Provenance

SYNTHETIC - synthetic-v1 - image 141d5d2e0810...

Predicted grade

Grade 2

Confidence / model score

70.0%

Probability distribution - Grades 0-4

Grade 0

5.0%

Grade 1

10.0%

Grade 2

70.0%

Grade 3

10.0%

Grade 4

5.0%

SYNTHETIC_FIXTURE_NOT_MODEL_INFERENCE

PRISM-DR - Localized visual evidence

Bounded clinician-facing lesion suggestions are shown without changing raw provider output.

Model id

mock-lesion

Revision

synthetic-v1

Task

lesion-roi

Runtime / status

remote - ASSET_REQUIRED

Preprocessing

synthetic-fixture-v1

Provenance

SYNTHETIC - synthetic-v1 - image 141d5d2e0810...

Raw lesion count

Bounded review count

1

1

Lesion count by class

MICROANEURYSM

Score information

Bounded score range 80.0-80.0%

Localized visual evidence

Scroll to zoom - drag to pan - double-click to fit

100%

Fit

Viewer 80%

All overlay geometry remains in original image pixels.

AI suggestion

Human annotation

SYNTHETIC_FIXTURE_NOT_MODEL_INFERENCE

Case lineage & annotation provenance

Read-only evidence for this case. Hashes identify the source and exact analysis bytes; counts do not establish accuracy.

Source SHA-256

141d5d2e0810b2b77cd6f44ee62cb1c93bbb97267cb0705f7fef64d6bb021

Analysis SHA-256

Not recorded

AI visual evidence

1

Human annotations

0

CVAT imported

0

Inference metadata

mock-global

22/09/2026, 18:24:15

synthetic-v1

mock-lesion

22/09/2026, 18:24:15

synthetic-v1

AI outputs support clinician review and are not ground truth.

Suggestions support review. They do not establish a diagnosis or autonomous referral.

Figure 6.2: Models & Audit

22

7 Troubleshooting and quick reference

Observation	Action
Unsupported modality	The DICOM is recognized but is not supported ophthalmic fundus input. Do not treat it as a generic decoder error or send it to the models.
AI analysis is not available	Check the Model API URL, /health, firewall, token configuration, and service readiness. Local Worklist and human review can remain available.
Model score needs interpretation	Treat it as model evidence, not a calibrated probability. Human review remains authoritative.
No PRISM overlays	Check whether Show AI suggestions is enabled, the class filter, and the model result. An empty result does not prove lesion absence.
Draft save failed	Keep the page open, correct the reviewer or connectivity issue, and use Save draft again. The unsaved guard is intentional.
Case is not Lesion-ready	Confirm the current active annotation set and verify that its hash has not changed since Confirm Annotation .
Case is not DR-ready	Check image admission/quality, final grade, reviewer, timestamp, and grade provenance. Annotation confirmation is not required for DR-ready.
CVAT unavailable	Continue local review and export where possible. Retry only after the configured URL, project, credentials, and network are available.

7.0.1 Privacy and safety limitations

Use only approved data. Keep tokens, raw PHI, DICOM private headers, model weights, and runtime databases out of screenshots, exports, and source control. The product is a research and clinical-support proof of concept, not an autonomous or regulatory decision system.

7.0.2 Quick reference

Need	Page / action
Admit or exclude a case	Worklist
Confirm context	Confirm Image
Inspect image and optional AI	Review
Record final grade	Clinician Review > Confirm DR Grade
Correct/remove an AI ROI	Edit Annotations
Finalize active annotation set	Confirm Annotation
Check output eligibility	Datasets
Inspect provenance	Models & Audit

Need	Page / action
Set reviewer name or hints	Settings